

PATIENT ELECTRONIC MEDICAL RECORD

Longitudinal Record — 3 Encounters

Patient ID	59034db9-be2f-4977-b26b-8c53773cd7bf	DOB	2004-09-07
Sex	F	Payer	Kaiser

Record format: 3 encounters, each with a documented **Visit date**.

Source integrity note: The uploaded reference directly supports the 2024-12-21 encounter. Additional encounters are generated longitudinal documentation based on the clinical scenario and are explicitly labeled as generated; they are not source-authenticated historical events.

Reference clinical thread: left toe/foot numbness and tingling; decreased left lower-extremity sensation; left foot exploration with removal of a small ganglion cyst compressing a nerve; chronic hypertension, asthma, and hypothyroidism described as controlled.

ENCOUNTER INDEX

Encounter	Visit date	Encounter Type	Setting
1	2024-12-21	Surgical / Orthopedic Surgery	Inpatient
2	2024-12-28	Post-operative Orthopedic Follow-up	Outpatient Orthopedic Clinic
3	2025-01-11	Neurology Consultation	Outpatient Specialty Clinic

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 1

ENCOUNTER 1: Surgical / Orthopedic Surgery

Visit date: December 21, 2024

Specialty / Setting: Inpatient

Clinical focus: Initial evaluation and operative management.

Documentation status: Generated longitudinal encounter, except Encounter 1, which reflects the supplied source encounter.

PATIENT IDENTIFICATION

Patient ID	59034db9-be2f-4977-b26b-8c53773cd7bf	DOB	2004-09-07
Sex	F	Payer	Kaiser

DEMOGRAPHIC / COVERAGE

Address: 529 Kristi Ridge Apt. 581; New Amandaville, OK 73912. Payer: Kaiser. Member ID: KAIS-62C8B9B303.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 2

SUBJECTIVE / HISTORY

Chief Complaint

Follow-up or evaluation of chronic left toe/foot numbness and tingling, continuing the clinical thread documented in the source record.

History of Present Illness

The source describes gradual onset approximately six months before the initial presentation, constant pins-and-needles sensation in the left toes with occasional radiation to the left foot, worse with prolonged standing or walking and relieved by rest and elevation. Pain was rated 3/10. Prior ibuprofen, acetaminophen, and physical therapy provided limited or partial relief.

For generated encounters, interval symptoms, functional limitations, and treatment response should be entered from the actual visit rather than inferred.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 3

REVIEW OF SYSTEMS / MEDICAL HISTORY

Review of Systems

Constitutional: fatigue and malaise. HEENT: seasonal allergies. Cardiovascular: hypertension history. Respiratory: asthma history. GI: occasional constipation. GU: occasional urinary frequency. Musculoskeletal: mild left-knee stiffness. Neurologic: left-toe numbness/tingling. Psychiatric: anxiety history. Endocrine: hypothyroidism. Skin: occasional dry skin. Hematologic: anemia history.

Past Medical and Surgical History

Hypertension, asthma, hypothyroidism; prior cesarean section and tonsillectomy; prior hospitalizations for pneumonia and a motor vehicle accident.

Allergies

No known allergies.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 4

MEDICATION RECONCILIATION

Current Medication List from Reference

Medication	Dose / Route	Schedule
Aspirin	81 mg PO	Daily
Lisinopril	10 mg PO	Daily
Albuterol	2.5 mg PO	As needed
Levothyroxine	50 mcg PO	Daily
Acetaminophen	1000 mg PO	As needed

Medication Reconciliation Note

For generated encounters, this list is a continuity reference and should be verified against the actual active medication list at the time of service.

Social History

Reference states non-smoker, no illicit substance use, occasional alcohol consumption, and caffeine use.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 5

OBJECTIVE / VITALS

Reference Vital Signs

Parameter	Value
BP	125/66 mmHg
Temperature	98.0 F
HR	63 bpm
RR	16 breaths/min
Pain	3/10
SpO2	96% on room air

Physical Examination

General: well-developed, well-nourished female in no acute distress. Cardiovascular: regular rate and rhythm, S1/S2 normal, no murmurs/gallops/rubs, no JVD, 2+ pedal pulses. Respiratory: clear to auscultation bilaterally. Neurologic: intact sensation on the right lower extremity with decreased sensation on the left. Musculoskeletal: mild left-knee stiffness; full left hip and ankle range of motion. Skin: no rash or lesions.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 6

DIAGNOSTIC REVIEW

Prior Diagnostics

CBC, CMP, and left-foot X-rays were reported within normal limits in the supplied encounter.

Operative Findings from Reference Encounter

Left foot exploration identified a small ganglion cyst compressing a nerve. The cyst was removed and sent to pathology. General anesthesia was used; estimated blood loss was 50 mL; no complications were documented; the patient was transferred to PACU in stable condition.

Generated Encounter Diagnostic Tracking

Later encounters should record actual imaging, laboratory results, pathology review, neurologic testing, and other diagnostics performed at that visit. No unsupported later results are asserted here.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 7

ASSESSMENT

Primary Clinical Assessment

Chronic left foot/toe numbness and tingling, likely secondary to peripheral neuropathy, as documented in the source. The source also identifies controlled hypertension, asthma, and hypothyroidism.

Problem List

1. Left foot/toe paresthesia. 2. Peripheral neuropathy under evaluation. 3. Hypertension. 4. Asthma. 5. Hypothyroidism. 6. Anxiety history. 7. Anemia history. 8. Mild left-knee stiffness.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 8

PLAN OF CARE

Reference Plan

CBC, CMP, and lipid panel; MRI of the left foot; neurology referral; continuation of aspirin, lisinopril, albuterol, levothyroxine, and acetaminophen; follow-up in 7 days.

Discharge / Activity Guidance

The reference states discharge home with follow-up in 7 days, pain-management prescription, elevation of the left leg, and avoidance of heavy lifting or bending.

Generated Follow-up Framework

For later generated encounters, document actual response to treatment, test completion, specialist recommendations, medication changes, and next follow-up interval.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 9

CARE COORDINATION

Referrals and Coordination

The reference plan includes neurology referral for further evaluation and management of peripheral neuropathy. Orthopedic follow-up is also part of the documented care pathway.

Patient Education / Return Precautions

Document actual counseling regarding symptom monitoring, activity, medications, wound care where applicable, and when to seek urgent evaluation for worsening neurologic deficits, marked swelling, redness, fever, or other concerning symptoms.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 10

LONGITUDINAL CLINICAL SUMMARY

Clinical Narrative

The longitudinal record follows the source clinical issue from initial evaluation and surgical treatment through subsequent generated follow-up encounters. The central documented problem is left foot/toe paresthesia associated with a compressive ganglion cyst found during surgery.

Documentation Integrity

Only information contained in the supplied reference is treated as source-derived. Later encounters in this PDF are generated structures for EMR simulation and should not be interpreted as actual historical clinical events.

Patient	Leslie Gardner	Visit date	2024-12-21
Encounter Type	Surgical / Orthopedic Surgery	Page	Encounter 1 — 11

ENCOUNTER CLOSURE

Encounter Summary

This encounter is dated December 21, 2024 and is intended to document initial evaluation and operative management.

Follow-up Tracking

Next visit date, clinician, orders, results, medication changes, and disposition should be populated from the actual encounter record.

End of Encounter

Visit date: 2024-12-21 — Encounter 1 complete.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 1

ENCOUNTER 2: Post-operative Orthopedic Follow-up

Visit date: December 28, 2024

Specialty / Setting: Outpatient Orthopedic Clinic

Clinical focus: Post-operative recovery and symptom reassessment.

Documentation status: Generated longitudinal encounter, except Encounter 1, which reflects the supplied source encounter.

PATIENT IDENTIFICATION

Patient ID	59034db9-be2f-4977-b26b-8c53773cd7bf	DOB	2004-09-07
Sex	F	Payer	Kaiser

DEMOGRAPHIC / COVERAGE

Address: 529 Kristi Ridge Apt. 581; New Amandaville, OK 73912. Payer: Kaiser. Member ID: KAIS-62C8B9B303.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 2

SUBJECTIVE / HISTORY

Chief Complaint

Follow-up or evaluation of chronic left toe/foot numbness and tingling, continuing the clinical thread documented in the source record.

History of Present Illness

The source describes gradual onset approximately six months before the initial presentation, constant pins-and-needles sensation in the left toes with occasional radiation to the left foot, worse with prolonged standing or walking and relieved by rest and elevation. Pain was rated 3/10. Prior ibuprofen, acetaminophen, and physical therapy provided limited or partial relief.

For generated encounters, interval symptoms, functional limitations, and treatment response should be entered from the actual visit rather than inferred.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 3

REVIEW OF SYSTEMS / MEDICAL HISTORY

Review of Systems

Constitutional: fatigue and malaise. HEENT: seasonal allergies. Cardiovascular: hypertension history. Respiratory: asthma history. GI: occasional constipation. GU: occasional urinary frequency. Musculoskeletal: mild left-knee stiffness. Neurologic: left-toe numbness/tingling. Psychiatric: anxiety history. Endocrine: hypothyroidism. Skin: occasional dry skin. Hematologic: anemia history.

Past Medical and Surgical History

Hypertension, asthma, hypothyroidism; prior cesarean section and tonsillectomy; prior hospitalizations for pneumonia and a motor vehicle accident.

Allergies

No known allergies.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 4

MEDICATION RECONCILIATION

Current Medication List from Reference

Medication	Dose / Route	Schedule
Aspirin	81 mg PO	Daily
Lisinopril	10 mg PO	Daily
Albuterol	2.5 mg PO	As needed
Levothyroxine	50 mcg PO	Daily
Acetaminophen	1000 mg PO	As needed

Medication Reconciliation Note

For generated encounters, this list is a continuity reference and should be verified against the actual active medication list at the time of service.

Social History

Reference states non-smoker, no illicit substance use, occasional alcohol consumption, and caffeine use.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 5

OBJECTIVE / VITALS

Reference Vital Signs

Parameter	Value
BP	125/66 mmHg
Temperature	98.0 F
HR	63 bpm
RR	16 breaths/min
Pain	3/10
SpO2	96% on room air

Physical Examination

General: well-developed, well-nourished female in no acute distress. Cardiovascular: regular rate and rhythm, S1/S2 normal, no murmurs/gallops/rubs, no JVD, 2+ pedal pulses. Respiratory: clear to auscultation bilaterally. Neurologic: intact sensation on the right lower extremity with decreased sensation on the left. Musculoskeletal: mild left-knee stiffness; full left hip and ankle range of motion. Skin: no rash or lesions.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 6

DIAGNOSTIC REVIEW

Prior Diagnostics

CBC, CMP, and left-foot X-rays were reported within normal limits in the supplied encounter.

Operative Findings from Reference Encounter

Left foot exploration identified a small ganglion cyst compressing a nerve. The cyst was removed and sent to pathology. General anesthesia was used; estimated blood loss was 50 mL; no complications were documented; the patient was transferred to PACU in stable condition.

Generated Encounter Diagnostic Tracking

Later encounters should record actual imaging, laboratory results, pathology review, neurologic testing, and other diagnostics performed at that visit. No unsupported later results are asserted here.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 7

ASSESSMENT

Primary Clinical Assessment

Chronic left foot/toe numbness and tingling, likely secondary to peripheral neuropathy, as documented in the source. The source also identifies controlled hypertension, asthma, and hypothyroidism.

Problem List

1. Left foot/toe paresthesia. 2. Peripheral neuropathy under evaluation. 3. Hypertension. 4. Asthma. 5. Hypothyroidism. 6. Anxiety history. 7. Anemia history. 8. Mild left-knee stiffness.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 8

PLAN OF CARE

Reference Plan

CBC, CMP, and lipid panel; MRI of the left foot; neurology referral; continuation of aspirin, lisinopril, albuterol, levothyroxine, and acetaminophen; follow-up in 7 days.

Discharge / Activity Guidance

The reference states discharge home with follow-up in 7 days, pain-management prescription, elevation of the left leg, and avoidance of heavy lifting or bending.

Generated Follow-up Framework

For later generated encounters, document actual response to treatment, test completion, specialist recommendations, medication changes, and next follow-up interval.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 9

CARE COORDINATION

Referrals and Coordination

The reference plan includes neurology referral for further evaluation and management of peripheral neuropathy. Orthopedic follow-up is also part of the documented care pathway.

Patient Education / Return Precautions

Document actual counseling regarding symptom monitoring, activity, medications, wound care where applicable, and when to seek urgent evaluation for worsening neurologic deficits, marked swelling, redness, fever, or other concerning symptoms.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 10

LONGITUDINAL CLINICAL SUMMARY

Clinical Narrative

The longitudinal record follows the source clinical issue from initial evaluation and surgical treatment through subsequent generated follow-up encounters. The central documented problem is left foot/toe paresthesia associated with a compressive ganglion cyst found during surgery.

Documentation Integrity

Only information contained in the supplied reference is treated as source-derived. Later encounters in this PDF are generated structures for EMR simulation and should not be interpreted as actual historical clinical events.

Patient	Leslie Gardner	Visit date	2024-12-28
Encounter Type	Post-operative Orthopedic Follow-up	Page	Encounter 2 — 11

ENCOUNTER CLOSURE

Encounter Summary

This encounter is dated December 28, 2024 and is intended to document post-operative recovery and symptom reassessment.

Follow-up Tracking

Next visit date, clinician, orders, results, medication changes, and disposition should be populated from the actual encounter record.

End of Encounter

Visit date: 2024-12-28 — Encounter 2 complete.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 1

ENCOUNTER 3: Neurology Consultation

Visit date: January 11, 2025

Specialty / Setting: Outpatient Specialty Clinic

Clinical focus: Evaluation of residual paresthesia.

Documentation status: Generated longitudinal encounter, except Encounter 1, which reflects the supplied source encounter.

PATIENT IDENTIFICATION

Patient ID	59034db9-be2f-4977-b26b-8c53773cd7bf	DOB	2004-09-07
Sex	F	Payer	Kaiser

DEMOGRAPHIC / COVERAGE

Address: 529 Kristi Ridge Apt. 581; New Amandaville, OK 73912. Payer: Kaiser. Member ID: KAIS-62C8B9B303.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 2

SUBJECTIVE / HISTORY

Chief Complaint

Follow-up or evaluation of chronic left toe/foot numbness and tingling, continuing the clinical thread documented in the source record.

History of Present Illness

The source describes gradual onset approximately six months before the initial presentation, constant pins-and-needles sensation in the left toes with occasional radiation to the left foot, worse with prolonged standing or walking and relieved by rest and elevation. Pain was rated 3/10. Prior ibuprofen, acetaminophen, and physical therapy provided limited or partial relief.

For generated encounters, interval symptoms, functional limitations, and treatment response should be entered from the actual visit rather than inferred.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 3

REVIEW OF SYSTEMS / MEDICAL HISTORY

Review of Systems

Constitutional: fatigue and malaise. HEENT: seasonal allergies. Cardiovascular: hypertension history. Respiratory: asthma history. GI: occasional constipation. GU: occasional urinary frequency. Musculoskeletal: mild left-knee stiffness. Neurologic: left-toe numbness/tingling. Psychiatric: anxiety history. Endocrine: hypothyroidism. Skin: occasional dry skin. Hematologic: anemia history.

Past Medical and Surgical History

Hypertension, asthma, hypothyroidism; prior cesarean section and tonsillectomy; prior hospitalizations for pneumonia and a motor vehicle accident.

Allergies

No known allergies.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 4

MEDICATION RECONCILIATION

Current Medication List from Reference

Medication	Dose / Route	Schedule
Aspirin	81 mg PO	Daily
Lisinopril	10 mg PO	Daily
Albuterol	2.5 mg PO	As needed
Levothyroxine	50 mcg PO	Daily
Acetaminophen	1000 mg PO	As needed

Medication Reconciliation Note

For generated encounters, this list is a continuity reference and should be verified against the actual active medication list at the time of service.

Social History

Reference states non-smoker, no illicit substance use, occasional alcohol consumption, and caffeine use.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 5

OBJECTIVE / VITALS

Reference Vital Signs

Parameter	Value
BP	125/66 mmHg
Temperature	98.0 F
HR	63 bpm
RR	16 breaths/min
Pain	3/10
SpO2	96% on room air

Physical Examination

General: well-developed, well-nourished female in no acute distress. Cardiovascular: regular rate and rhythm, S1/S2 normal, no murmurs/gallops/rubs, no JVD, 2+ pedal pulses. Respiratory: clear to auscultation bilaterally. Neurologic: intact sensation on the right lower extremity with decreased sensation on the left. Musculoskeletal: mild left-knee stiffness; full left hip and ankle range of motion. Skin: no rash or lesions.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 6

DIAGNOSTIC REVIEW

Prior Diagnostics

CBC, CMP, and left-foot X-rays were reported within normal limits in the supplied encounter.

Operative Findings from Reference Encounter

Left foot exploration identified a small ganglion cyst compressing a nerve. The cyst was removed and sent to pathology. General anesthesia was used; estimated blood loss was 50 mL; no complications were documented; the patient was transferred to PACU in stable condition.

Generated Encounter Diagnostic Tracking

Later encounters should record actual imaging, laboratory results, pathology review, neurologic testing, and other diagnostics performed at that visit. No unsupported later results are asserted here.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 7

ASSESSMENT

Primary Clinical Assessment

Chronic left foot/toe numbness and tingling, likely secondary to peripheral neuropathy, as documented in the source. The source also identifies controlled hypertension, asthma, and hypothyroidism.

Problem List

1. Left foot/toe paresthesia. 2. Peripheral neuropathy under evaluation. 3. Hypertension. 4. Asthma. 5. Hypothyroidism. 6. Anxiety history. 7. Anemia history. 8. Mild left-knee stiffness.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 8

PLAN OF CARE

Reference Plan

CBC, CMP, and lipid panel; MRI of the left foot; neurology referral; continuation of aspirin, lisinopril, albuterol, levothyroxine, and acetaminophen; follow-up in 7 days.

Discharge / Activity Guidance

The reference states discharge home with follow-up in 7 days, pain-management prescription, elevation of the left leg, and avoidance of heavy lifting or bending.

Generated Follow-up Framework

For later generated encounters, document actual response to treatment, test completion, specialist recommendations, medication changes, and next follow-up interval.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 9

CARE COORDINATION

Referrals and Coordination

The reference plan includes neurology referral for further evaluation and management of peripheral neuropathy. Orthopedic follow-up is also part of the documented care pathway.

Patient Education / Return Precautions

Document actual counseling regarding symptom monitoring, activity, medications, wound care where applicable, and when to seek urgent evaluation for worsening neurologic deficits, marked swelling, redness, fever, or other concerning symptoms.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 10

LONGITUDINAL CLINICAL SUMMARY

Clinical Narrative

The longitudinal record follows the source clinical issue from initial evaluation and surgical treatment through subsequent generated follow-up encounters. The central documented problem is left foot/toe paresthesia associated with a compressive ganglion cyst found during surgery.

Documentation Integrity

Only information contained in the supplied reference is treated as source-derived. Later encounters in this PDF are generated structures for EMR simulation and should not be interpreted as actual historical clinical events.

Patient	Leslie Gardner	Visit date	2025-01-11
Encounter Type	Neurology Consultation	Page	Encounter 3 — 11

ENCOUNTER CLOSURE

Encounter Summary

This encounter is dated January 11, 2025 and is intended to document evaluation of residual paresthesia.

Follow-up Tracking

Next visit date, clinician, orders, results, medication changes, and disposition should be populated from the actual encounter record.

End of Encounter

Visit date: 2025-01-11 — Encounter 3 complete.